

MRI SCAN OF BRAIN

MRI No:2169/25

Mr Viraj Ranjan

54 yrs

Date of the Study: 08-09-2025

Clinical History: R/ facial numbness, blurred vision, ptosis.

Referring Clinician: Dr Yohan Koralege (Consultant Neurosurgeon)

Technique: Non-contrast study with T1w, T2w, FLAIR axials, T1, T2 sagittal, T2 coronal, DWI, ADC, SWI and intracranial MR angiogram performed.

Findings:

There is no acute infarct or other diffusion-restricting lesion. No old infarcts or encephalomalacia.

No abnormal parenchymal signal intensity identified. **No brain stem lesions suggestive or demyelination.**

The thin T2 3D images of IAM region show bony stenosis at the proximal inlet of right IAM with impingement of right facial nerve.

Left IAM appear normal.

No compressive lesions identified at basal cisterns. Trigeminal nerves appear normal.

The ventricular system shows normal calibre for age. Cortical sulci and basal cisterns are normal.

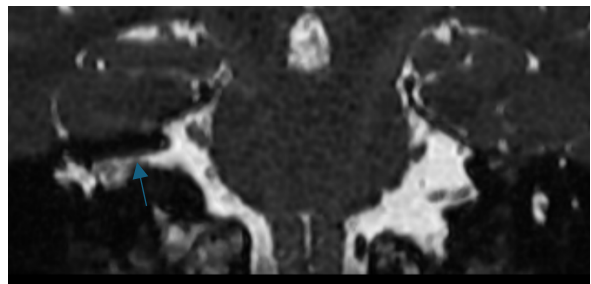
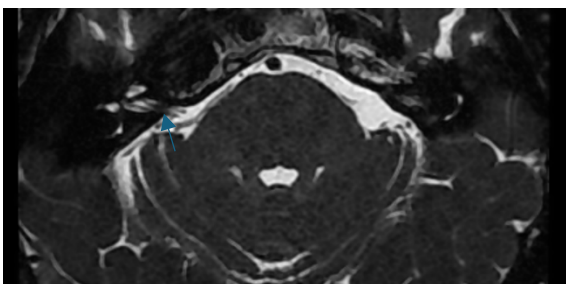
The midline commissural structures, sella, pineal region appear normal. Bilateral basal ganglia and thalami are normal.

No cerebellar or brain-stem abnormality was identified.

No intra or extra-axial mass lesion or mass effect.

No intra or extra-axial collection or haematoma. No evidence of remote parenchymal or pial haemorrhage.

Major intracranial arteries and dural venous sinuses return normal flow voids in visualised images.



Bilateral maxillary sinus basal mucosal thickening. No otomastoid effusion.

Impression:

- Impingement of right facial nerve at the inlet of IAM. (Although congenital bony canal stenosis of IAM is a rare cause for facial nerve symptoms, considering this patient's age, this being the cause for facial numbness is controversial. ? Incidental finding)
- No acute infarcts, demyelinating lesions or other intracranial abnormalities identified.

Reported by:.....

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